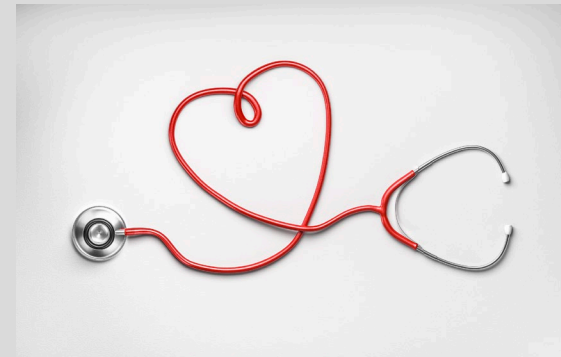


Session Objectives

- Introduce Prevention and cardiac risk factors found in history and physical exam
- Discuss pitfalls of communication
- Discuss methods of documenting risk factors in assessment and addressing it in the plan.
- Discuss the role of communication (written and verbal) in expressing your decision-making process including the information you are trying to communicate

Source: Course Syllabus



Question 1

- A 55-year-old asymptomatic black male presents to the clinic.

He is concerned that his brother recently had a heart attack at age 57. He denies any symptoms of chest pain, palpitations, dyspnea, nor syncope. His father had heart disease and passed away in his 60's. He does not smoke, his BMI is 38, and his blood pressure is 140/90. After completing a complete history and physical exam, how would you end this encounter?

- A. Recommend that he lose weight.
- B. Recommend that he should eat a healthy diet.
- C. Assess other cardiac risk factors including glucose and cholesterol.
- D. Advise him that he should exercise more
- E. Advise him that it is genetics and there is nothing he can do but take medicines.

Think about what in the patient's history is cardiovascular risk factors?

Cardiovascular Risk Factors

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- First degree relatives with CVD: *found in family history*
- Tobacco and excessive alcohol use: *found in social history*
- Physical Inactivity: *found in social history*
- Poor Diet: *found in social history*
- BMI; Central Obesity: *found with vitals and exam*
- Hypertension: *found with vitals*
- Dyslipidemia: *ordered in labs*
- Diabetes/Fasting blood glucose: *ordered in labs*
- Presence of Atherosclerosis/Existing CVD: *found in diagnostics , sometimes exam.*

Question 2

A 46-year-old male diabetic returns to the clinic for follow up on elevated blood pressure. You have previously counselled the patient on improving their diet, increasing aerobic exercise, and weight loss. Over the last month the patient has managed to lose 5 lbs and the blood pressure has lowered from 160/100 to 135/90 today. Which is the next step?

- a. Start blood pressure medicine since diastolic is 90 and lifestyle modification failed.
- b. Continue current plan since systolic is less than 140.
- c. Evaluate and modify the current plan to better achieve goal of bp <140/90.
- d. Recommend the *Carnivore Diet*.
- e. Encourage tobacco use as it will suppress the appetite.

Before you get started

- Things to consider:

What is apparent?

- Age
- Vitals
- Chief complaint

What possible risk factors could the patient have?

Tip: Prepare for your history and physical exam. Establish routine in note taking and organization of notes.

Written Documentation Purpose

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- Communication:
 - In hospital help other teams understand what you are doing.
 - In clinic help you remember what you are doing.
 - At home- patient understands what is going on.
- Billing:
 - Aid insurance companies with evidence to support your efforts and to not commit Fraud.
- Legal:
 - Aid Lawyers with evidence to support how you addressed the patient's illness and concerns.

**DECISION
MAKING
PROCESS**

Social history

- Social: tobacco/alcohol/drugs.
- *Documenting previous and past use helps the reader understand risk. Risk of cardiovascular disease or even cancer.*

Example:

- Patient smoked 1ppd cigarettes for 30 years but quit 15 years ago. It's great that they stopped but still has risk of cancer and CVD.

Case: History Documented

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- PMH: Hypertension
- PSX: denies
- Meds: Hydrochlorothiazide 25mg daily. (didn't take today)
- Allergies: Penicillin: rash, coconut- itching
- FH:
 - Mom deceased age 80 stroke
 - Dad deceased age 82 due to MI
 - Brother 63 alive hypertension
 - Denies children
- Social:
 - Tobacco 1ppd/30 years. Quit 15 years ago.
 - Alcohol: 1 beer/day for most of adulthood
 - Denies illicit drug use, denies formal exercise.
 - Caffeine- 1 cup coffee a day
 - Diet: high salt and fast food.

New Updates ACC/AHA

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- Apolipoprotein B
 - Test once LDL-C and non-HDL-C goals are met
- Lipoprotein (a)
 - Measure at least once to identify those at higher risk.

Assessment

- Assessment helps the reader know what conditions you:
 - Considered (differential diagnosis)
 - Prioritized (your #1)
 - Addressed

Giving rationale helps the reader understand your decision-making process.

You should also consider risk factors in your assessment.

Case: Assessment

- DDX: Chest Pain
 1. Costochondritis: started with activity and worsened with movement. Exam elicit upon palpation.
 2. Angina: multiple risk factors: hypertension, tobacco history, family history, unknown cholesterol.
 3. Pleurisy: worse with breathing

Assessments:

1. Hypertension
2. Physical Inactivity
3. Unhealthy diet

Plan Documentation:

- Inform/Educate/Counsel
 - Do not just document: “I counselled on diet”
 - It is not specific, informative to reader, or give an understanding of patient’s response.
 - Document: “I recommended the patient to eat a DASH diet based on her hypertension. I described the diet and the patient voiced understanding.”

Documenting Tobacco cessation:

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I recommended the patient to quit smoking due to the increased risk it brings for heart and lung disease and cancer. Options given included medications, patches, cutting back, and counseling. Patient voiced understanding of risk and states they will work on cutting back. Follow up 1 month to see how it is going.

Written Decision Making: Recap

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- As you follow your note from top to bottom/subjective to plan, the reader should be able to track:
 - Presentation including pertinent positives / negatives within the HPI.
 - Medical history/risk factors that could include or exclude potential for a condition.
 - Objective findings in the vitals, exam, or labs that may help confirm or deny a condition is present.
 - An assessment that connects to the subjective and objective data including differential diagnosis of the chief complaint as well as other presenting conditions.
 - A Plan that directly connects to the assessment. Make sure any recommendation/order has a direct assessment tie-in.

Verbal

- Examples:
 - SBAR, handoffs, Oral Presentation
- Organized. Give timely information so that:
 - Listener can determine what is going on with the patient.
 - Attending can follow your decision-making process as well as form a differential diagnosis while listening.

Oral Presentation Details

- Details about the case is important
 - Including the elements of HPI
 - Including risk factors
- But not all details need to be stated.
 - Examples: non relevant.
 - History that doesn't pertain to the case.
 - However- some history that is negative is relevant to help form differential.
 - Example: lack of existing conditions, lack of risk factors.

Case example:

Remember: Organization is key

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Give name, demographics, chief complaint, then HPI, PMH, PSX, meds, allergies, Pertinent Family history, and social risk factors.

- Example: In this case would include past history of smoking.

Give vitals, general appearance you focused exam of heart/lungs/OSE and palpation findings.

Give your differential of Costochondritis and what you recommended.